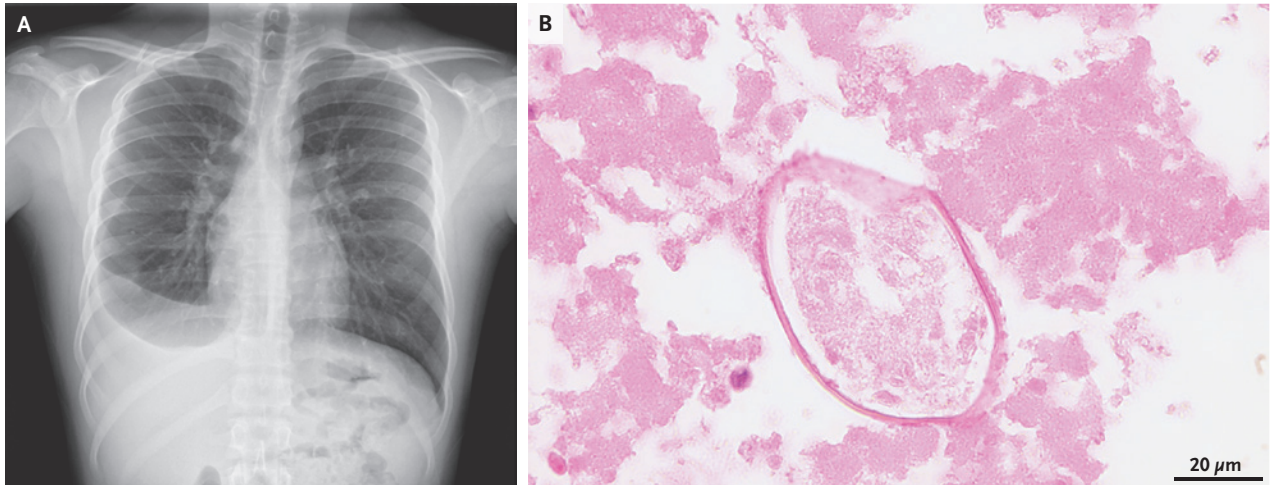


IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Pleural Paragonimiasis



A N ASYMPTOMATIC 17-YEAR-OLD BOY WHO HAD EMIGRATED FROM EL Salvador 6 months earlier was found to have a pleural effusion on a chest radiograph obtained during a routine assessment at an immigration clinic (Panel A). A physical examination was normal. Computed tomography of the chest also showed the effusion, as well as a 4-mm solid peripheral lung nodule on the same side. Blood tests showed eosinophilia. Findings from analysis of the pleural fluid were consistent with an exudate. Eosinophils accounted for 13% of the nucleated cells in the pleural fluid (reference value, <10%). A small number of parasite eggs were seen in the pleural fluid (Panel B, hematoxylin and eosin staining). Targeted 28S ribosomal RNA sequencing of the pleural fluid detected *Paragonimus mexicanus*. Testing for tuberculosis was negative. A diagnosis of pleural paragonimiasis was made. Paragonimiasis, a lung fluke infection, is acquired through consumption of undercooked crustaceans, which the patient recalled having eaten 2 years before emigrating. Paragonimiasis is often asymptomatic, but any symptoms are typically pulmonary in nature owing to parenchymal involvement. *P. westermani*, which is endemic to Asia, is the most common species that infects humans, but *P. mexicanus* infection occurs in Central and South America. The patient received treatment with praziquantel. At a 4-month follow-up visit, the size of the effusion had decreased minimally.

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